Reg. No. RMEE2552994
PURE PATH LAB
Add: Infront of Cantabil, Beside Distt. Hospital-Gonda
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PURE PATH LAB
Palce of Accuracy & Your Satisfaction
Dr. Abha Mishra
M.B.B.S (M.D. PATH)
Reg. No-MP-8779

Mr. Rupesh Pandey
Age / Sex : 25 YRS / Male
Reg. no. : 6997

Registered on : -
Collected on : -
Received on : -
Reported on : -

HAEMATOLOGY

TEST		VALUE	UNIT	REFERENCE
HEMOGLOBIN		14.9	g/dl	13 - 17
TOTAL LEUKOCYTE COUNT		7600	cumm	4,800 - 10,800
NEUTROPHILS	**H**	**83**	**%**	**40 - 80**
LYMPHOCYTE	**L**	**12**	**%**	**20 - 40**
EOSINOPHILS		03	%	1-6
MONOCYTES		02	%	2 - 10
BASOPHILS		00	%	<2
PLATELET COUNT	**L**	**1.45**	**lakhs/cumm**	**1.5 - 4.1**
TOTAL RBC COUNT	**L**	**4.32**	**million/cumm**	**4.5 - 5.5**
HEMATOCRIT VALUE, HCT	**L**	**39.5**	**%**	**40 - 50**
MEAN CORPUSCULAR VOLUME, MCV		91.4	fL	83 - 101
MEAN CELL HAEMOGLOBIN, MCH	**H**	**34.5**	**Pg**	**27 - 32**
MEAN CELL HAEMOGLOBIN CON, MCHC	**H**	**37.7**	**g/dl**	**31.5 - 34.5**
MEAN PLATELET VOLUME, MPV	**H**	**12.9**	**fL**	**6.5 - 12**
R.D.W. - SD		42.1	fL	39 - 46
R.D.W. - CV		13.4	%	11.6 - 14
P-LCR	**H**	**47.2**	**19.7**	**- 42.4**
P.D.W.	**H**	**16.6**	**fL**	**9.6 - 15.2**

Possible causes of abnormal parameters

	High	Low
RBC, Hb, or HCT	Dehydration, polycythemia, shock, chronic hypoxia	Anemia, thalassemia, and other hemoglobinopathies
MCV	Macrocytic anemia, liver disease	Microcytic anemia
WBC	Acute stress, infection, malignancies	Sepsis, marrow hypoplasia
Platelets	Risk of thrombosis	Risk of bleeding

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BIOCHEMISTRY

TEST		VALUE	UNIT	REFERENCE
SERUM BILIRUBIN (TOTAL)		0.91	mg/dl	0.2 - 1.2
SERUM BILIRUBIN (DIRECT)		0.18	mg/dl	0 - 0.3
SERUM BILIRUBIN (INDIRECT)		0.73	mg/dl	0.2 - 1
SGOT (AST)		33.57	U/l	0 - 37
SGPT (ALT)	**H**	**45.87**	**U/l**	**13 - 40**
SERUM ALKALINE PHOSPHATASE		125.8	U/l	44 - 147
SERUM PROTEIN		6.44	g/dl	6.4 - 8.3
SERUM ALBUMIN	**L**	**3.28**	**g/dl**	**3.5 - 5.2**
GLOBULIN		3.16	g/dl	1.8 - 3.6
A/G RATIO	**L**	**1.04**	**1.1**	**- 2.1**

LFT Interpretation

Liver Function Blood Test gives an insight into your liver health and helps identify problems like hepatitis, cirrhosis, and fatty liver disease, which may cause similar symptoms but require different treatments to recover.

Test Significance

Besides diagnosing liver problems, LFT's also monitor overall liver functioning. Monitoring helps people with liver disease or taking medication, as it helps screen whether the treatment works fine or requires adjustments. Moreover, Liver Function Tests help determine if someone is at risk of developing liver diseases. Apart from assessing your chances, this test also checks the severity of the liver damage to help the doctor plan and prescribe appropriate treatment.

Increased in

Acute or chronic hepatitis, cirrhosis, biliary tract obstruction, toxic hepatitis, neonatal jaundice (neonatal hyperbilirubinemia), congenital liver enzyme abnormalities (Dubin-Johnson, Rotor, Gilbert, Crigler-Najjar syndromes), fasting, hemolytic disorders. Hepatotoxic drugs.

RANDOM BLOOD SUGAR		125.9	mg/dl	70 - 140
SERUM UREA		38.6	mg/dl	19 - 45
SERUM CREATININE		1.08	mg/dl	0.72 - 1.43
SERUM PHOSPHORUS	**H**	**4.54**	**mg/dl**	**2.5 - 4.5**
SERUM URIC ACID		7.15	mg/dl	3.5 - 7.2
SERUM SODIUM		136.1	mmol/L	136 - 146
SERUM POTASSIUM		4.56	mmol/L	3.5 - 5.1
ICALCIUM		1.15	mmol/l	1.13 - 1.33

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Creatinine is a nitrogenous waste product formed in muscle from creatine phosphate. Endogenous production of creatinine is proportional to muscle mass and body weight.

Causes of Increased Serum Creatinine Level	Causes of Decreased Serum Creatinine Level
1. Pre-renal, renal, and post-renal azotemia	1. Pregnancy
2. Large amount of dietary meat	2. Increasing age (reduction in muscle mass)
3. Active acromegaly and gigantism	

~~~ End of report ~~~
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